



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Policy and Procedure

Perinatal Mental Health Liaison Service Referral

Reference #: FSH-MENH-GUI-0004

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Mental Health Service Women's and Newborn Service	Medical, Nursing, Midwifery and Allied Health

1. Introduction

Fiona Stanley Hospital provides tertiary maternity care for women ranging from low obstetric risk to women with complex medical and obstetric histories, who reside within the South Metropolitan catchment area. The perinatal period is recognised as a high-risk period for women with mental health disorders.

The FSH Perinatal Mental Health Liaison Service (PNMHLS) is an outpatient and inpatient Consultation Liaison service that supports Women's and Newborn Services (WaNS) to identify, assess and treat mothers at risk of poor perinatal mental health outcomes. This policy will outline the processes for screening and referral to the PNMHLS for mental health assessment and development of a mental health plan including treatment and support as indicated.

2. Terminology

Perinatal period	Period from pregnancy until 12 months after childbirth.
e-Referral	Referral or electronic consultation via an electronic platform to enable the seamless transfer of patient information from a primary to a secondary treating practitioner.
Edinburgh Postnatal Depression Scale (EPDS)	A ten-question screening tool used to screen women at risk for 'perinatal' depression.

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The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

Psychosocial factors	Include social supports, mental illness, and family and domestic violence, history of trauma, alcohol or substance use.
Community Midwifery Program (CMP)	A model of care available to Perth women identified as low risk for pregnancy and birth who are interested in having a home birth.
Midwifery Led Care	Midwives primarily responsible for care: refer and discuss as required with Medical Practitioner.
Obstetric Led Care	Medical Practitioner led care (for women assessed as not appropriate for low-risk midwifery care)
Acute Severe Behavioural Disturbance (ASBD)	Behaviour that puts the patient or others at immediate risk of serious harm and may include threatening or aggressive behaviour, extreme distress, and serious self-harm which could cause major injury or death.

3. Policy

- In line with the [Australian Pregnancy Care Guidelines](#), women attending for antenatal care will be screened for mental health risk factors and symptoms using the EPDS and psychosocial screening.
- Women attending for antenatal care who screen positive should be referred for further mental health assessment and development of a perinatal mental health treatment and support plan as early as practicable.
- The referring FSH WaNS maternity care team retain primary clinical responsibility for the woman's clinical treatment.
- The PNMHLS provide a consultation and liaison service to the referrer and patient regarding the woman's mental health care during the episode of maternity care.
- A woman can be re-referred to the PNMHLS at any time during their episode of maternity care.

4. Procedure

4.1. Referral Criteria

4.1.1. Inclusion Criteria:

- Currently active with FSH WaNS
- Screen positive by EPDS (EPDS >13 or Q10 scores of one, two or three) and/or psychosocial screening for mental health symptoms or risk factors

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- (Repeat EPDS if not completed in the previous 2 weeks)
- Mental health concerns (past and/or present)
 - Current mental health symptoms:
 - Current concerns of risk to self or baby due to mental health symptoms
 - Impaired mental state e.g. presenting with confusion, manic or psychotic features/strange behaviour
 - Current destabilisation of mental health (e.g. worsening depression and/or anxiety)
 - Past mental health history:
 - Major psychiatric illness (e.g. Psychosis, Schizophrenia, Bipolar Disorder, Depression, Anxiety, Obsessive Compulsive Disorder, Post Traumatic Stress Disorder, Eating Disorder or Personality Disorder)
 - History of postpartum psychosis
 - History of suicide attempts/ self-harming behaviours

4.1.2. Exclusion criteria:

- Women not currently active with FSH WaNS
- Women not referred during FSH WaNS episode of care

4.2. Referral Process

PNMHLS is available Monday – Friday 08:00 – 16:30hrs.

Urgent referrals occurring out of these hours must be medically reviewed by obstetric team prior to contacting on call psychiatric team for urgent mental health assessment.

Non urgent referrals will be reviewed by the PNMHLS team the next working day.

- Patients have the right to decline PNMHLS referral. Referrals to PNMHLS should indicate if the patient has consented or not.
- Patients who consent will be triaged.
- Triage patients who meet PNMHLS criteria will be offered an appointment for mental health assessment and development of a perinatal mental health treatment and support plan.

- For patients who do not agree to PNMHLS engagement, a referral can be made for information only. The PNMHLS team may provide advice and guidance to the referring team.
- If a woman requires an urgent mental health assessment and refuses consent for the PNMHLS team to review their mental health, request the obstetric team medically review the patient. The PNMHLS team will provide advice and support as needed.
- The e-Referral form to the PNMHLS should include criteria to determine the urgency of response:
 - Urgent referrals are recommended for acute mental health symptoms with high agitation or distress – PNMHLS will review within an appropriate timeframe according to acuity.

For acutely distressed reviewed PNMHL the PNMHLS will:

- clarify the patient's level of immediate safety
- recommend an appropriate level of supervision for the patient and baby
- consider the need for emergency management ASBD. Refer to the [KEMH Mental health emergency, Management of Clinical Practice Guideline](#)

- Moderately urgent referrals - An appointment will be offered within one to two weeks.
- Non urgent/routine referrals - An appointment will be offered within four to six weeks.
- An e-Referral must be completed for all PNMHLS referrals:
 - Psychiatry [PSY] □ Liaison Service – Perinatal [MHPN]
 - Phone the PNMHLS CNS to verbally advise of **all inpatient referrals** and **immediate outpatient** referrals to ensure a timely response to the referral.

4.3. Referral Triage

- Referrals are triaged each working day by the PNMHLS CNS/CN to establish clinical need and urgency.
- Referrals will be prioritised based on the:
 - Relevant Information provided by referrer including reason for referral and current mental health concerns and risks
 - Consultation with the patient and PNMHLS psychiatrist

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- Referral outcomes are documented in the patient's medical record and e-Referral

4.4. Referral Management

- The PNMHLS will provide assessment information and recommendations to the treating team regarding the PNMHL care plan documented in the patient's medical record.
- For women referred to the PNMHLS who are uncontactable or do not engage, the PNMHLS will inform the referring Multidisciplinary Team (MDT):
 - Outpatient midwifery led care: liaise with the Midwifery Manager – Ambulatory with recommendations. Obstetric consultation may be clinically indicated in high-risk cases.
 - Obstetric led outpatient clinic: liaise with the Obstetric team with recommendations.
 - Inpatient referrals: liaise with the admitting team with recommendations.
- Women considered 'patients of concern': the PNMHLS team will communicate with the treating team and recommend presentation of the case at the WaNS Complex Care Meeting.
- PNMHLS will assist in facilitating early discharge where possible by reviewing inpatient referrals and liaising with the admitting inpatient WaNS team.
- For inpatients requiring PNMHLS consultation, timing of discharge by the WaNS team will be in consultation with the PNMHLS team.
- For inpatients referred by WaNS to the PNMHLS who are discharged by the WaNS team prior to PNMHLS team review, the referral will be declined. Clinical responsibility for the patient's treatment remains with the referring team (WaNS Midwifery and/or Obstetric led care.)
- If community mental health care is required following discharge, the WaNS treating team will make the necessary referrals.

5. Compliance/Performance Monitoring

- The PNMH Clinical Nurse Specialist will be responsible for monitoring compliance with this document. Compliance will be monitored via routine clinical incident review processes.
- A referrer feedback survey will be conducted yearly by the PNMHLS to ascertain utility of referral process, with provision of education and/or review/changes to referral process as indicated.

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6. Related Policy Documents

- WA Health:
 - [Consent to Treatment Policy 2016.](#)
- FSFHG:
 - Antepartum care (FSH-OBS-GUI-0002)
 - Patient Decision Making Capacity: Mental Health Consultation Liaison Input (FSFH-HW-GUI-0009)
 - Mental State Deterioration and Escalation Care Pathway (Adult) (FSFH-HWPOL-0081)
 - Clinical Handover (FSFH-HW-POL-0035)

7. Related Documents

- [COPE-Perinatal-MH-Guideline_Final-2018.pdf](#)
- [CAHS](#) Perinatal and Infant mental health

8. Related Legislation

- [Mental Health Act 2014](#)

9. Related Standards

- NSQHS Standards
 - Clinical Governance
 - Partnering with Consumers
 - Comprehensive Care
- Chief Psychiatric Standards for Clinical Care
 - Assessment
 - Care Planning

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10. Authorisation

EXECUTIVE SPONSOR:					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	04/22	CNS, MHS	NMPGC	S5 SQR Committee	04/2025
1.1	07/2024	Policy Team		FSFHEC	11/2024
2.0	07/2025	PNMHLS CNS	NMPGC	FSFHG Policy Committee	07/2028